



Pediatric Acute Stabilization Bronchiolitis: Outpatient Pathway

Inclusion Criteria

Age <2 years

Viral upper respiratory symptoms & lower respiratory symptoms that may include increased work of breathing, cough, feeding difficulty, tachypnea, wheeze, fever

Risk Factors for Severe Disease

- Cardiac disease requiring baseline medication
- Anatomic airway defects
- Neuromuscular disease
- Immunodeficiency
- Chronic lung disease

Infants with these risk factors present early in the illness have higher risk of progression: Gestational age < 34 weeks; Respiratory rate ≥ 70 ; Age < 3 months

Virtual Care Triage:
Video Visit Preferred

Same Day Office Visit:

- ☐ Wheezing with mild retractions
- ☐ Coughing with gagging or vomiting
- ☐ Decreased feeding but still >50% of normal intake

Urgent Care/ED:

- ☐ Moderate Retractions
- ☐ RR: <3moa: 61-80
RR: 3--<12moa: 51-70/min
RR: 1-2yoa: 41-60/min
- ☐ Wheezing with retractions
- ☐ Decreased feeding <50% of normal intake

Call 911:

- ☐ Severe Respiratory Distress
- ☐ Extreme irritability/agitation (does not want to be touched)
- ☐ Unable to swallow liquids
- ☐ Apnea/Cyanosis

MILD DISEASE

Alert, Active, Feeding, well

Minimal retractions (subcostal or abdominal)

RR: <3moa: 30-60/min

RR: 3--<12moa: 25-50/min

RR: 1—2yoa: 20-40/min

O₂ sat > 93% on Room Air

MODERATE DISEASE

Alert, Consolable, Feeding decreased (but still >50% of normal intake)

Min-moderate (subcostal & intercostal) retractions

RR: <3moa: 61-80

RR: 3--<12moa: 51-70/min

RR: 1-2yoa: 41-60/min

O₂ sat < 90-93% on Room Air

SEVERE DISEASE

Fussy, Difficult to console, Poor feeding (<50% of normal intake)

Moderate – severe retractions (subcostal, intercostal, suprasternal, nasal flaring)

RR: <3moa: >80 /min

RR: 3--<12moa: >70/min

RR: 1-2yoa: >60/min

O₂ sat < 90% on Room Air

Initial Assessment AND Interventions

- ☐ Assess SpO₂ – Provide supplemental O₂ to keep saturation > 90%
- ☐ Determine severity (Mild, Moderate, or Severe)
- ☐ Suction- Use bulb suction or BBG-olive tip with suction machine
- ☐ Consider Point of Care Glucose test – for hypoglycemia related to feeding (if oral intake <50%) or hyperglycemia related to stress or DKA
- ☐ Consider one-time trial of Albuterol for patients with:
 - Severe respiratory distress
 - Patients with increased risk of asthma (>12 months old with wheezing- Risk factors for asthma, history of atopy, recurrent wheezing or strong FH of atopy and/or asthma)



Reassess patient and response to interventions

Suction every 1 hour + prn
Feed if safe for oral feeds

**Pediatric Transfer
and Advice Center:**
562-658-3980 OR
8-137-3980

Discharge home

- Maintaining SpO₂ > 90% on room air
- Hydrated and able to feed at least 50% of normal feeds
- Mild/moderate work of breathing (improved or stable)
- Reliable caretaker
- Able to obtain follow-up care
- MDI/spacer teaching if response to albuterol

Early escalation of:

- Apnea with bradycardia + cyanosis
- Toxic appearance
- Signs of imminent respiratory failure

Call PTAC and Connect with Pediatric Hospitalist if:

- Risk factors for severe disease (noted above)
- RR consistently > 60
- Significant retractions and /or
- SpO₂ < 90% on R/A
- Dehydration or unable to feed

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References

1. Ralston, S. L., A. S. Lieberthal, H. C. Meissner, B. K. Alverson, J. E. Baley, A. M. Gadomski, D. W. Johnson, et al. "Clinical Practice Guideline: The Diagnosis, Management, and Prevention of Bronchiolitis." PEDIATRICS 134, no. 5 (November 1, 2014): e1474–1502. <https://doi.org/10.1542/peds.2014-2742>.
2. Children's Hospital of Philadelphia Clinical pathway: <https://www.chop.edu/clinical-pathway/bronchiolitis-emergent-evaluation-clinical-pathway>